

NHS Devon Integrated Care Board Service Specification

Service Specification No.	
Service	Community FeNO Testing v5
Commissioner Lead	
Provider Lead	
Period	1 st December 2025 – 31 st March 2029
Date of Review	

1. Population Needs

1.1 National/local context and evidence base

Respiratory disease is the third leading cause of death in England and a major contributor to health inequalities, emergency admissions, and winter pressures on the NHS. The NHS Long Term Plan and Core20PLUS5 framework identify chronic respiratory disease as a key priority area for reducing health disparities and improving outcomes.

Implementation of community-based FeNO testing ensures timely, equitable access to respiratory diagnostics. Early and accurate diagnosis of respiratory disease is associated with better patient outcomes, reduced morbidity, hospitalisations and reduced spend on unnecessary medications.

The FeNO Implementation Toolkit (2023) provide clear standards for quality-assured delivery, including training, accreditation, and data reporting.

This service is expected to be delivered at PCN level, to reduce variation and improve access.

Devon faces significant challenges in respiratory health:

- Higher-than-average prevalence of asthma and COPD, particularly in coastal and rural communities.
- Inconsistent access to diagnostic testing across the population
- Devon ICB's Joint Forward Plan (2025–2030) prioritises early diagnosis, prevention, and community-based care, with respiratory disease identified as a key area for transformation.

The Community FeNO Testing LES aligns with Devon's strategic goals by:

- Supporting early and accurate diagnosis in primary care.
- Reducing avoidable referrals and hospital admissions.

- Addressing inequities in access to respiratory diagnostics across the county.

2. Outcomes

2.1 NHS Outcomes Framework Domains & Indicators

Domain 1	<u>Preventing people from dying prematurely</u> Reducing deaths in babies and young children	✓
Domain 2	<u>Enhancing quality of life for people with long-term conditions</u> <u>Health related quality of life for people with long term conditions</u> <u>Ensuring people feel supported to manage their condition</u> <u>Improving functional ability in people with long term conditions</u>	✓
Domain 3	<u>Helping people to recover from episodes of ill-health or following injury</u> Emergency admissions for acute conditions that should not usually require hospital admission	✓
Domain 4	<u>Ensuring people have a positive experience of care</u> Patient experience of primary Care Patient experience of hospital care Friends and family Test <u>Improving children and young people's experience of healthcare</u> <u>Improving peoples experience of integrated care</u>	✓
Domain 5	<u>Treating and caring for people in safe environment and protecting them from avoidable harm</u> Patient safety Incidents reported Reducing the incidence of avoidable harm	✓

Local Defined Outcome

Patients are provided with a service in line with the locally agreed pathway and standards of care. The service delivered will be subject to one clinical audit per annum and annual review of the commissioned services in general practice, the structure of which will be developed collaboratively with the LMC.

3. Scope

3.1 Aims and objectives of service

- Improve early and accurate diagnosis of respiratory disease in general practice

- Reduce variation in access to community respiratory diagnostics testing.
- Support appropriate initiation and optimisation of treatment
- Reduce avoidable referrals to secondary care by managing patients closer to home.
- To address health inequalities in respiratory care across Devon, particularly in underserved populations.
- Promote healthier well-being within the community setting

3.2 Service description/care pathway

Provision of FeNO Testing at PCN level in the Community.

Each PCN will have the autonomy to determine where and when FeNO testing is delivered in Primary Care.

PCNs may work together in hub arrangements where this is mutually agreed and meets the needs of the registered populations of each PCN. Devon ICB will consider contracting with other groupings of GP practices in order to arrange optimal coverage of the service for the population

PCNs will:

- Deliver Fractional Exhaled Nitric Oxide (FeNO) testing to support the diagnosis and management of asthma, following NICE NG245.
- Provide clinical interpretation of results with a written report for the referring clinician.
- Make onward referral where appropriate (e.g. severe asthma services, respiratory outpatients).
- Provide an Annual Review to include:
 - Brief details as to the arrangements for FeNO testing
 - Details of training and education received by FeNO practitioners
 - Details of the standards of calibration and maintenance of FeNO machines

3.3 Process:

- Referral from GP/ANP with clinical indication.
- Where capacity allows and by mutual agreement, PCNs will accept referrals for FeNO from PCNs that are unable to undertake FeNO testing in-house.
- FeNO performed by a trained healthcare professional.
- Results to be interpreted by a qualified clinician.
- Report returned to referrer within 3 working days of test being undertaken.

3.4 Clinical Training

The Provider will demonstrate robust quality assurance processes which ensure that all clinicians undertaking FeNO testing are fully trained and competent

Staff performing FeNO must have completed the Health Education England eLearning for Healthcare, FeNO In Asthma eLearning programme (2 modules)

[FeNO in Asthma - eLearning for healthcare](#)

Whilst there is no recommended minimum activity level for FeNO Testing, it is recommended that FeNO testing competency is recorded using the South West Cardiovascular, Respiratory and Diabetes Clinical Network template included in the Fractional Exhaled Nitric Oxide (FeNO) Guidance for Primary Care (p8). [South West Respiratory Network FeNO Guidance](#)

Clinicians interpreting results must have documented training and maintain CPD in respiratory diagnostics.

All staff must complete annual competency assessments and participate in peer review.

3.5 Equipment and IT

NICE approved FeNO Monitors must be used.

Funded set-up costs include the provision of one FeNO Monitor per PCN where required. (see Section 7)

Test results must be coded and uploaded to the patient record using SNOMED codes (shown at Appendix 1).

Data must be shared with Devon ICB for quarterly review which should include as a minimum:

- Number of appropriate FeNO referrals received
- Number of inappropriate FeNO referrals received
- Number of FeNo referrals performed
- Number and reason of inconclusive FeNO tests e.g. patient unable to do the test or the test was equivocal
- Number of DNA referrals
- Diagnostic Outcomes
- Patient Postcode (to monitor equity of access)

3.6 Infection control

Services must follow local infection prevention guidance.

3.7 Clinical Governance

Providers must ensure:

- FeNO operators are trained in device use and interpretation.
- Regular calibration, servicing, and quality control of equipment in accordance with manufacturer's guidance

- Data recording in line with Information Governance requirements (results uploaded to GP record in line with good clinical record keeping standards).
- Clinical oversight by a named lead clinician.
- Incident reporting – Devon ICB should be informed immediately if the untoward incident will or has impacted on patient safety
- Participation in audit.

3.8 Population covered

All Devon GP registered patients (aged >5 years) within participating PCNs, with suspected Asthma or Chronic obstructive pulmonary disease.

3.9 Any acceptance and exclusion criteria and thresholds

FeNO testing through this LES will be provided to patients who meet the following criteria:

Patients aged >5 years with suspected diagnosis of one of the following listed respiratory conditions:

- Asthma
- Chronic obstructive pulmonary disease (COPD)
- Previously diagnosed with asthma who are suspected of having developed further respiratory disease (eg. COPD)

3.10 Exclusions:

- Children aged <5 years
- All contraindications for FeNO
- Haemoptysis of unknown origin
- Unstable cardiac conditions
- Suspected lung cancer or any other lung disease not specified in the specification e.g. ILD, TB etc.

4. Applicable Service Standards

Providers and practitioners will adhere to the local and national guidelines or any updated versions thereof.

NICE guidelines relating to equipment calibration, testing methodology and interpretation of results will be followed i.e.:

4.1 Applicable national standards (eg NICE)

FeNO - [NICE Asthma Guidelines \(Nov 2024\) NG245](#)

Applicable standards set out in Guidance and/or issued by a competent body (e.g. Royal Colleges)

FeNO - [Fractional Exhaled Nitric Oxide \(FeNO\) Guidance for Primary Care](#)

4.2 Applicable local standards

Not applicable

Equality and Diversity:

The service must conform to legislation prohibiting discrimination and good practice in relation to equality, diversity and inclusion.

Complaints and commendations:

Participating practices are required to establish and operate a complaints and commendations procedure in line with NHS guidelines to deal with any complaints in relation to any matter connected with the provision of services. All complaints should be monitored, audited and appropriate action taken when required.

5. Applicable quality requirements and CQUIN goals

5.1 Applicable Quality Requirements (See Schedule 4 Parts [A-D])

Each practice must ensure that any personnel involved in providing any aspect of care under this scheme has the necessary training, skill and competency to do so. A Registered Nurse will supervise any care delivered by Healthcare Assistants.

5.2 Applicable CQUIN goals (See Schedule 4 Part [E])

Not applicable

6. Location of Provider Premises

The provider will provide this service from any location as included in GMS, PMS or APMS contract where these facilities enable the provider to ensure privacy and dignity and appropriate infection control.

7. Funding for FeNO Tests

Tariff (Cost per Test):

- FeNO Test **£18.61** per test including interpretation

Indicative Activity levels for FeNO testing will be agreed with each PCN and reviewed throughout the year

The LES Tariff (Cost per Test) includes payments for :

FeNO:

- Staff Costs (admin and clinical)
- Consumables and Calibration
- Service Charge (Utilities, Insurance and Maintenance of equipment)

Payment will be made per completed test, subject to submission of required data and compliance with governance standards.

Set Up Costs

A FeNO Monitor will be provided by Devon ICB where required (1 per PCN, up to £2,395)

8. Activity Coding

Activity Coding :

FeNO activity must be recorded using the code below to receive payment.

SnoMed Code:

Measurement of Expired Nitric Oxide 444642008